Nursing Essentials

Priority: Assess progress alongside maternal / fetal well-being. Centimeters alone do not describe the whole labor.

FOCUS 1

Track progress

- **First:** labor onset to complete dilation. The active phase generally begins at 6 cm.
- **Second:** complete dilation to birth. **Third:** birth to placental delivery.
- **Immediate recovery:** reassess bleeding, uterine tone, vital signs, pain, and bladder function after birth.

FOCUS 2

Monitor labor

- **Before starting:** verify indication, consent, presentation, baseline assessment, and the ordered monitoring plan.
- **Oxytocin:** use a controlled pump and monitor uterine activity / fetal response; respond to tachysystole or concerning patterns.
- **Amniotomy:** assess fetal status before and after; report a sudden prolonged deceleration or possible cord concern.

FOCUS 3

Support analgesia

- **Monitor:** blood pressure, fetal status, pain relief, motor / sensory effects, and bladder needs.
- **Hypotension:** prompt positioning and ordered treatment support maternal and fetal perfusion.
- **Safety:** follow mobility precautions; help with position changes and reassess after interventions.

Assisted birth and follow-up

Make it stick: Labor progress is interpreted with the clinical context. Avoid diagnosing arrest from a single cervical examination or an outdated active-phase threshold.

Focus	What to know	Nursing application
Preparation	verify the plan, support informed care, and prepare maternal and newborn resources.	Prepare mother and newborn
After operative vaginal birth	assess maternal trauma / bleeding and the newborn for injury.	Reassess after birth
Newborn warning	enlarging diffuse scalp swelling, pallor, or poor perfusion after vacuum use may indicate subgaleal hemorrhage.	Escalate possible hemorrhage

RED FLAGS · ESCALATE PROMPTLY

Escalate concerning fetal changes, severe hypotension, increasing maternal bleeding, or enlarging newborn scalp swelling with poor perfusion.